CJC-1295 + Ipamorelin (CJC-1295 + NNC 26-0161)

Step-by-Step Reconstitution Process

- **Gather supplies: BAC water, alcohol swabs, insulin syringe, vial**
- Bring BAC water and peptide vial to room temperature before mixing
- **Removing plastic cap.** Carefully pop off the plastic cap, do not remove metal flange.
- **Clean both vials:** Wipe the rubber stoppers with alcohol swabs
- **Draw BAC water:** Pull your chosen amount into the syringe
- **Inject slowly:** Add the water to the peptide vial, aiming it at the glass wall—not directly onto the powder
- **Swirl gently:** Roll the vial between your palms. Never shake.
- **Wait:** Let it sit 5-10 minutes. Solution should be crystal clear.
- **Store:** Refrigerate immediately. Good for 28-30 days once reconstituted.

If it's cloudy or won't dissolve: Let it sit longer. Try gentle warming (under your arm or in lukewarm water). If still cloudy after 30 minutes, contact your source.

Practical Protocols

The following protocols are not derived from published dose-finding studies. They represent clinical practice patterns and community consensus.

None

STANDARD PROTOCOL

CJC-1295 (no DAC): 100 to 300 mcg per day

Ipamorelin: 100 to 300 mcg per day

Timing: Once daily, fasted (at least 2 hours after

eating)

Preferred timing: Before bed on an empty stomach

Schedule: Daily

Cycle length: 12 to 16 weeks, then 4 weeks off

None

BY BODY WEIGHT

Under 150 lbs: 100 to 150 mcg of each peptide

150 to 200 lbs: 200 mcg of each peptide

200 to 250 lbs: 250 to 300 mcg of each peptide

Over 250 lbs: 300 mcg of each peptide

None

SPLIT DOSING (ADVANCED)

Morning (fasted): 100 to 150 mcg of each

Before bed: 100 to 150 mcg of each

This creates two separate growth hormone pulses per day.

None

RECONSTITUTION (standard blend vial)

Vial size: 10 mg total (5 mg CJC-1295 no DAC + 5 mg

Ipamorelin)

Bac water: 2 mL

Concentration: 250 mcg of each peptide per 10 units on insulin

syringe

At 250 mcg of each per day, one vial lasts approximately 20 days.

Why Fasting Matters

When you eat, your blood sugar rises and your body releases insulin. Insulin binds directly to the cells in your pituitary gland that produce growth hormone and suppresses their ability to release it. If you inject these peptides while insulin is present in your blood, you are pressing the gas and the brake at the same time. Take the injection at least 2 hours after eating to ensure insulin has cleared.

Timing Based on Your Goal

Once growth hormone is released, it travels to your liver where it gets converted into IGF-1. But your liver needs insulin present to efficiently convert growth hormone into IGF-1. This creates a timing consideration.

If your primary goal is fat loss, injecting before bed makes the most sense. Growth hormone sits in your system overnight and directly mobilizes fat without needing IGF-1 for that process. You sleep fasted, and the growth hormone works on fat burning all night.

If your primary goal is muscle building, consider injecting in the morning about 30 minutes before you eat. You get the full growth hormone release because you are fasted, then your meal provides the insulin needed to convert it into IGF-1 for muscle protein synthesis. The fasting requirement stays exactly the same. What changes is what you do after you inject.

Receptor Desensitization: The 5 On / 2 Off Myth

The old recommendation was 5 days on, 2 days off to prevent receptor desensitization. This protocol has no scientific basis and sits in a useless middle ground that addresses neither form of actual desensitization.

Acute desensitization occurs within minutes of each dose and resolves in approximately 60 minutes. A study using GHRP-6 in perifused rat pituitary cells found that receptor sensitivity fully recovered after just 60 minutes. Two days off does nothing for this because it resolves on its own within an hour of each injection.

Chronic desensitization develops after approximately 16 weeks of continuous use and requires about 4 weeks off to fully reverse. A human study gave 12 elderly subjects twice-daily hexarelin injections for 16 weeks. Growth hormone response declined significantly by week 16 but returned to baseline after 4 weeks off. Two days off is far too short to reverse this type of desensitization.

The tesamorelin trials that received FDA approval used daily dosing for 26 weeks. The MK-677 trials used daily dosing for two years. In both cases, growth hormone and IGF-1 stayed elevated the entire time.

The 5 on / 2 off protocol likely started with old-school growth hormone protocols when GH was expensive and people took days off to stretch their supply. Then clinics started charging premium prices for peptides, and taking two days off makes a 30-day supply last 6 weeks instead of four, which makes the business model work better for the clinic. The protocol stuck because it benefited the business model, not because the science supported it.

Daily dosing is the evidence-based approach. If you want to prevent chronic desensitization, run continuously for up to 16 weeks followed by 4 weeks off. If you want to stretch your supply on a tight budget, dosing 5 days per week saves about 30% on product cost. That is a valid budget strategy, but understand it has nothing to do with desensitization.

hormone directly, your IGF-1 levels rise, and your hypothalamus releases somatostatin in response. Somatostatin blocks your pituitary's ability to respond to GHRH signals. Research showed that after a single injection of exogenous growth hormone, the GHRH pathway was substantially inhibited and the ghrelin pathway was also blunted. The secretagogues you are paying for cannot do their job because somatostatin is blocking the signal at the pituitary. Secretagogues and exogenous GH are different tools designed for different goals. Secretagogues work within your body's natural limits and keep your feedback loop intact. Exogenous GH or IGF-1 bypasses the feedback loop entirely.

If you are transitioning from exogenous IGF-1 to this blend, allow 2 to 3 days for the IGF-1 to clear and the negative feedback to begin reversing.

Common Questions

Is CJC-1295 without DAC the same as Mod GRF 1-29? Yes. They are the same compound. CJC-1295 without DAC and Mod GRF 1-29 are two names for the same modified GHRH analog. The original CJC-1295 was designed with DAC for extended duration. The no-DAC version is what is used in the blend because it preserves natural pulsatile release.

Do I need to split the dose or can I take it all at once? Once daily works, but splitting gives better results because you catch two natural growth hormone peaks. If you are going to pick one time, bedtime fasted is the best option because it amplifies the largest natural growth hormone pulse that occurs during deep sleep.

Can I use CJC-1295 by itself without ipamorelin? You can, but you are leaving results on the table. CJC alone activates only the GHRH pathway. Adding ipamorelin activates the ghrelin pathway simultaneously, which is where the synergistic effect comes from. CJC alone will still elevate growth hormone, just not to the same degree as the combination.

Should I choose this blend or tesamorelin? It depends on your goal. Both work through the GHRH pathway, but CJC + ipamorelin hits two pathways while tesamorelin hits only one. For overall optimization including sleep, recovery, and muscle support, the blend is the better choice because of the dual-pathway synergy. Tesamorelin has clinical trial data specifically for visceral fat reduction, so if stubborn belly fat is your main concern and you want the compound with the most targeted data for that outcome, tesamorelin is worth considering. But the mechanism is the same GHRH pathway that CJC already covers.

How long do I need to run this to see results? Expect sleep improvements in weeks 1 to 2. Recovery improvements in weeks 2 to 4. Body composition changes around weeks 8 to 12. Connective tissue benefits at months 3 to 6. Most people who are disappointed with the blend stopped too early.

Can I take it every day or do I need days off? Daily dosing is the evidence-based approach. The 5 on / 2 off protocol has no scientific support. Run continuously for up to 16 weeks, then take 4 weeks off if you are concerned about chronic desensitization.

Will this replace TRT for building muscle? No. Growth hormone peptides do not replace testosterone for building muscle. They are recovery and optimization tools. If you are looking for anabolic muscle growth, testosterone is the primary driver. This blend supports that process but does not replace it.

Side Effects

What the Research Shows

CJC-1295 was well tolerated in clinical trials at doses of 30 to 60 mcg/kg. No serious adverse reactions were reported. Common side effects were transient injection site reactions, mild flushing, and headache.

Ipamorelin showed a clean side effect profile in human trials, with no significant elevations in cortisol, prolactin, or ACTH even at supraphysiological doses. This is the primary safety advantage over older ghrelin mimetics.

What Users Report

Common (usually resolve within 2 weeks):

- Injection site redness or irritation
- Water retention (mild)
- Tingling or numbness in fingers or hands (transient)
- Flushing shortly after injection
- Vivid dreams

• Mild lightheadedness (more common at higher doses)

Less common:

• Headache
• Mild increased hunger
• Fatigue during initial adaptation
• Temporary blood sugar fluctuations

Warning Signs That You Have Gone Too Far

If you notice persistent numbness or tingling in your hands, carpal tunnel symptoms, joint swelling, or facial puffiness from water retention, your dose is too high. Reduce the dose or stop temporarily. More is not better with growth hormone peptides. These symptoms are your body telling you the growth hormone elevation is exceeding what your system can handle cleanly.